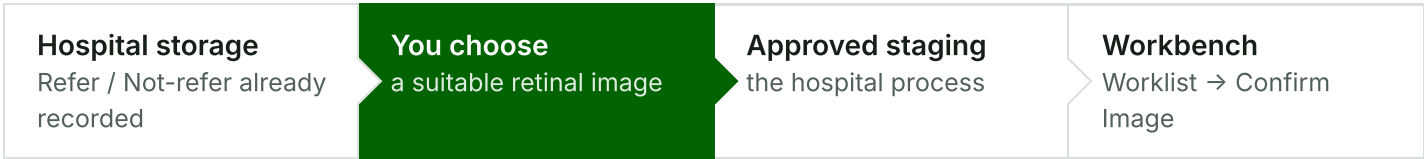


Choosing a retinal image for review

A few seconds

การเลือกภาพจอประสาทตาเพื่อส่งเข้า Workbench

per image, on top of your normal choice





Illustration

WHAT SHOULD I CHOOSE?

Use
ใช้ได้

- A colour **fundus / retinal photograph** already classed **Refer** or **Not-refer**.
- Enough retina is visible** for normal reading: optic disc, macula, vessels.
- The **original file**, staged through the approved hospital process.



Screenshot / other

WHAT SHOULD I AVOID?

Skip
ไม่ต้องส่ง

- Not a retinal photo: **OCT**, external-eye or other images.
- Screenshots**, crops, edited or annotated copies.
- Corrupt or unreadable files, or clearly unusable images.
- An image you know you **already staged**.



Blurred / dark

WHAT IF I AM UNSURE?

Review first
ไม่แน่ใจ ดูอีกครั้ง

- Borderline blur or exposure, unsure which eye, or unsure enough retina shows: **take a second look**.
- Would you still use it clinically? Stage it. If not, skip it or ask the coordinator.
- Do not guess or invent details**. "Unknown" is fine.



No

DO I NEED TO MANAGE DATASET BALANCE MYSELF?

No — the team does this
ไม่ต้อง ทีมโครงการดูแลเอง

The project team monitors overall Refer/Not-refer, sex, and left/right coverage. Clinicians do not need to manage dataset quotas themselves.

No counting, no forms, no grading before review. If more of one kind is needed, the coordinator will say so for the whole batch.